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Health Guide PL

Getting public healthcare in Poland



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List of abbreviations::

AOS – Outpatient specialist services (Ambulatoryjna Opieka Specjalistyczna)

ePUAP - Electronic Platform of Public Administration Services (elektroniczna Platforma Usług Administracji Publicznej)

eZLA - Electronic sick note (elektroniczne Zwolnienie Lekarskie)

IKP - Online patient account (Internetowe Konto Pacjenta)

Karta DiLO - Card for Cancer Diagnosis and Treatment (Karta Diagnostyki i Leczenia Onkologicznego)

klub AA - Alcoholics Anonymous club (klub Anonimowych Alkoholików)

KOC - Coordinated care for pregnant women (Koordynowana Opieka nad Kobieta w ciąży)

NFZ - National Health Fund (Narodowy Fundusz Zdrowia)

POZ - Primary health care (Podstawowa Opieka Zdrowotna)

PUE ZUS - Electronic services platform of the Social Insurance Institution (Platforma Usług Elektronicznych Zakładu Ubezpieczeń Społecznych)

SOR - Casualty department (Szpitalny Oddział Ratunkowy)

USC - Register office (Urząd Stanu Cywilnego)

I. Public health care

1. What is the structure of health care in Poland?

In Poland, there is public health care, available to every insured person, financed from public funds, and paid health care. In our publication we will look at public health care.

Some facilities provide both paid and free health services, remember to make sure when registering for an appointment that it is the National Health Fund (NFZ)-contracted service - free of charge.

2. Am I entitled to free medical care?

Yes. Every person working in Poland is subject to compulsory health insurance. Health insurance aims to protect people's health. As part of the insurance, everyone is entitled to free treatment, diagnostics, nursing care, rehabilitation, hospital stay, spa and preventive health care. If you are insured in Poland, you have the same access to medical care as insured Polish citizens.

3. How can I get insurance?

If you are employed on the basis of a contract of employment or a contract of mandate, your employer will report you to the Social Insurance Institution (ZUS) for health insurance, and the insurance premiums will be paid from your salary. If you are self-employed, you must register for insurance with the Social Insurance Institution yourself. Contributions from ZUS are transferred to the

National Health Fund (NFZ). It is a state organisational unit for the financing of health services.

4. Can I insure my wife and children?

Register all non-working family members living with you - spouse, child, parents or grandparents - for health insurance. The registration of family members is free and unlimited (you can register as many family members as you wish).

5. How do I choose my doctor and clinic?

You can choose any Primary Health Care (POZ) facility in your province. It is most convenient if the facility is located close to your home. You will need to go to a clinic of your choice (known as a POZ facility) and fill in a declaration of choice for a doctor, nurse and, for women, a midwife.

Forms for declarations of choice can be found at the end of this publication. Declarations can also be made through your online patient account (IKP). You can change your POZ doctor twice a year for free.

You can find a list of POZ clinics on the NFZ website:

<https://gsl.nfz.gov.pl/GSL/GSL/>.

II. Primary Health Care (POZ)

1. Who is a POZ doctor?

The POZ care doctor treats adults (specialist in internal medicine or family medicine) and children (specialist in paediatrics or child diseases or specialist in family medicine).

2. What does a POZ doctor do?

The POZ doctor manages the entire treatment process. His/her remit includes:

- Treating patients and giving advice.
- Establish and monitor pharmacological treatment.
- Issuing prescriptions and orders for medical supplies.
- Issuing of sick notes.
- Referral for preventive vaccinations and periodic check-ups.
- Sending for tests, e.g. laboratory diagnosis, ECG, X-ray, ultrasound.
- Interpretation of test results.
- Referral to spa and rehabilitation treatment.
- Referral to specialist doctors and hospital.
- Referral to nursing long-term home care.
- Health adjudication.
- Issuing an order for the services of a POZ nurse.

3. How do I make an appointment with a POZ doctor?

Most often the appointment with the doctor is set by telephone, in special cases the appointment is made on the day of registration, in other cases on average within 3-5 days.

4. What documents do I need to take with me to the POZ appointment?

Always take with you an identity document with a photo and PESEL number, and in the case of a child, a medical record book. For your first appointment, take all your medical records from your treatment to date, and a list of the drugs you are taking. If it is your next appointment, take the results of any tests, medical consultations that your POZ has not yet seen.

You can find a short guide on how to prepare for a visit to POZ on this website:

https://www.nfz-szczecin.pl/jqt1c_zanim_pojdziesz_do_lekarza_rodzinnego.htm

5. Does the consultation of the POZ doctor always take place at the clinic?

When the patient's state of health prevents a visit to the clinic, the visit may be carried out at the place of residence by prior arrangement with the doctor. This is known as a home visit. If a problem does not require examination by a doctor, you can get

help over the phone. This is known as an online doctor consultation.

6. Can I also go to the POZ clinic with my child?

Yes. Your POZ clinic should have a paediatrician or family doctor who can diagnose and treat paediatric conditions and carry out preventive vaccination and well-child cares. Remember that in order for your child to receive services at a POZ clinic, you must make a declaration of choice of doctor.

7. Who is a POZ nurse?

The primary health care (POZ) nurse is a community nurse. You can make a declaration of choice by going to a POZ or via your online patient account.

The nurse cares for patients both in the clinic and at home. The tasks of the community nurse include:

- Drug administration.
- Performing injections.
- Removal of sutures.
- Performing inhalations.
- Carrying out nursing and therapeutic procedures.

8. Who is a POZ midwife?

The POZ midwife provides comprehensive nursing, obstetric and gynaecological care. The POZ midwife cares for women from 21 weeks of pregnancy and newborns. Up to 2 calendar months of

the child's life, the POZ midwife is obliged to make 4 - 6 patronage visits. You can meet the midwife at her office at the clinic or at your home, depending on the situation.

The tasks of the midwife include:

- Care for pregnant women.
- Recognising health problems.
- Planning and implementation of preventive services.
- Education of women with high-risk pregnancies.
- Care of newborns.
- Examination of children including assessment of general condition, adaptation of the child to the environment.
- Helping to create a bond between mother and child and to initiate breastfeeding.
- Promotion of parents' health-seeking behaviour.

9. What hours does POZ provide care?

You can go to the POZ clinic every day from Monday to Friday from 8:00 am to 6:00 pm. When your POZ clinic is closed, you can go to a night and holiday healthcare facility.

III. Online patient account

1. What is an online patient account (IKP)?

It is a tool that organises scattered medical information. Anyone with a PESEL has IKP, but only adults can use this account.

To use IKP you need to log on to the website:

<https://pacjent.gov.pl/>. For this you need an electronic signature, which can be a trusted profile, a temporary trusted profile or an e-ID card.

IKP provides patients with information about their past, current or planned treatment and allows them to deal with a range of issues without having to visit a clinic or advice clinic. It gives online access to your full medical history, the visits you have made to the clinic or doctor, the care you have received, and how much NFZ has paid for these services. It gives access to e-prescriptions, information on issued referrals and their status, e-sick notes and medical certificates issued in connection with illness and maternity. IKP also provides access to information on minor children (if you have insured them) and family members who have provided us with access to their health information. There is also a free app for your phone – "mojeikp".

2. How do I log into IKP and make a declaration to the POZ?

Step 1

Visit <https://pacjent.gov.pl/>

Step 2

In the upper right corner, click on the "Zaloguj się" button

Step 3

Once logged in, select "Moje konto" from the tab bar and under the tab find the "Twoja Podstawowa Opieka Zdrowotna (POZ)" title. Click and fill in the declarations.

Step 4

When you are presented with the option of filling in three e-declarations, start by choosing a POZ doctor. In the "Lekarz" field click "Wypełnij e-deklaracje" (Important! Remember to choose a doctor, a community nurse and, for women, a midwife for comprehensive medical care)

Step 5

Your data will be entered by itself. Check and complete them.

Step 6

Select the province, town, street and facility.

Step 7

After filling in all the fields, click on the right-hand side "Podpisz e-deklaracje". The declaration will be signed using a trusted profile or a qualified signature. You are now eligible for primary health care.

IV. Prescription

1. What is a prescription?

A prescription is a document issued by a doctor on the basis of which a pharmacist can sell you a particular drug.

2. What drugs are prescribed?

A prescription is usually needed to purchase antibiotics and medicines ordered by medical specialists that would be dangerous to use on their own. These include, for example, sleeping pills, sedatives and anti-epileptic drugs.

3. Which drugs can I buy without a prescription?

Basic painkillers and antipyretics, some medicines used to treat the common cold, some ointments can be purchased without a prescription.

4. How do I get a prescription?

The prescription is issued by a doctor, less often by a nurse, in a face-to-face visit or during online doctor consultation after taking a history and planning the treatment. The prescription can be a 4-digit PIN number or a printout with a barcode or QR code. PIN will be given by your doctor, but you also have it in your IKP, and if you select this option in your IKP, you will receive it by SMS or e-mail.

5. I have a prescription and what happens next?

You apply the prescription at the pharmacy by entering your PESEL number and 4-digit PIN number or by showing the barcode or QR code of the prescription.

6. How do I buy drugs at a discount?

Some medicines can be bought more cheaply through reimbursement. When issuing the prescription, the doctor determines the amount of the discount depending on the diagnosis he has made (chronic diseases, certain infections). For patients aged 75 and over, some drugs are free of charge.

7. Can my e-prescription become out of date?

Yes. The validity of the prescription may depend on the type of a drug. Most prescriptions are valid for 30 days.

Exceptions:

- a prescription for an oral or injectable antibiotic is valid for 7 days (in the case of antibiotics in ointments or drops, the prescription is normally valid for 30 days)
- a prescription issued for 365 days - medicines can be bought at one time or in several packs, but at the same pharmacy. If you buy your first pack later than 30 days, your pharmacist will reduce the number of packs. You can buy medicines for 180 days of therapy at a time.
- The prescription for immunological preparations individually prepared (e.g. vaccines for desensitisation) and imported from abroad is valid for 120 days. This does not apply to mass-produced vaccines - e.g. against influenza - such a prescription is only valid for 30 days.

V. Sick note

1. What is electronic sick note (eZLA)?

A doctor may issue a certificate justifying an employee's absence due to illness - so-called eZLA (commonly called L4 from the name of the form). The employer downloads the sick note in electronic form from his profile on the ZUS Electronic Services Platform (PUE ZUS). In rare cases, the employer does not use PUE ZUS, in which case the doctor will print out the sick note which you must provide to your employer within 7 days.

2. How long can I be on a sick note?

You are entitled to a maximum of 182 days of sick leave, the exception being sick leave during pregnancy or for tuberculosis - a maximum of 270 days.

3. What if I am no longer entitled to sick note but am still ill?

If you have exhausted the maximum number of days of sick leave and your entitlement to sick pay has run out, you can apply to the Social Insurance Office for rehabilitation benefit for 12 months.

4. Am I entitled to leave from work if a child or other family member is ill?

Yes. A doctor may issue a certificate justifying an employee's absence due to his or her illness or the illness of persons under his

or her care. An eZLA issued to a parent who cannot continue working because of the child's illness is a carer's benefit.

Annually can be used:

- 60 days of care for a sick child up to 14 years
- 14 days of care for a sick child aged 14-18 or other family member.
- 30 days for illness of disabled children aged 14 to 18.

5. Is the salary during leave the same as during work?

Sickness and carer's benefit is 80% of salary.

It can be 100% of salary if:

- it is the sickness benefit during pregnancy
- the illness is caused by an accident at work or on the way to or from work
- absence from work is due to participation in medical procedures foreseen for donors of cells, tissues and organs

6. Will my employer find out what I am ill with?

The eZLA form received by the employer does not contain a diagnosis of the illness. Sometimes the sick note will include one of the letter codes that specify the circumstances of the illness affecting the benefit.

A code - the inability to work has occurred after an interval of not more than 60 days, following the same disease which was the cause of the inability to work before the interval

B code - inability to work during pregnancy.

C code - inability to work due to alcohol abuse. An employee who has received sick note with the C code is not entitled to wages or benefits for the first 5 days from the date the sick note began.

D code - inability to work due to tuberculosis.

E code - inability to work due to an infectious disease that has an incubation period of more than 14 days or another disease whose symptoms manifest themselves after more than 14 days from the beginning of the disease.

On your written request, the doctor may choose not to enter codes B and D.

7. Can I get a back-dated or forward-dated sick note?

Usually the doctor will issue a sick note with the date of the appointment. A sick note can be written up to a maximum of 3 days back from the medical examination or online doctor consultation. An exception is a sick note from a psychiatrist, which can be written out at an even earlier date. The sick note can also be written forward with the date of the next day or, if the following days are public holidays, with the date on which you should be at work.

8. Do I need to do any surveys before returning to work?

If you have been ill for more than 30 days you need a certificate from your POZ doctor stating that you are able to return to work. After receiving this certificate, your employer must refer you to an occupational doctor for a check-up to determine whether you can work in the same position. The cost of the tests is covered by the employer.

VI. Primary health care – children.

1. What kind of care can my child expect at POZ?

Usually the doctor's work is divided into consultations of sick and healthy children. As part of the sick children's consultation, children with symptoms of infection are welcomed. During the reception hours for healthy children there are vaccinations, well-child cares and sometimes diagnostics for diseases that are not caused by an acute infection-e.g. abdominal pain, eating disorders, abnormal growth.

2. What is a well-child care?

It should be carried out in children aged 1-4 weeks, 2-6 months, 9 months, 12 months (on dates corresponding to preventive vaccinations), 2 years, 4 years, 5 years, one-year compulsory pre-school preparation, 3rd grade of primary school, 1st grade of lower

secondary school, 1st grade of upper secondary school, the last year of upper secondary school until the age of 19

Children who do not yet go to school have their well-child care done entirely in POZ. School-age children start their well-child care at the school nurse's office (weighing, measuring, blood pressure check, eye examination, posture check). There, after an initial examination, they receive a card with which they must go to the POZ clinic.

The doctor checks that height, weight pressure, posture, visual acuity are normal. He/she performs a general medical examination, evaluates the implementation of the preventive vaccinations. If necessary, he/she issues referrals to other specialists or laboratory tests.

3. How do I vaccinate my child?

Children staying in Poland for more than 3 months should continue to be vaccinated to match the vaccination calendar. To do this, you need to make an appointment with a paediatrician or family doctor at chosen POZ (where you have made your declarations). At the time of registration it should be indicated that this is a vaccination. For vaccination, it is necessary to prepare documents in which the previously administered vaccines are recorded (e.g. the child's medical record book).

4. What is the vaccination calendar?

The vaccination calendar is a list of compulsory vaccinations with an indication of the age at which they are due. It is updated annually by the Chief Sanitary Inspector.

5. How is my child vaccinated?

The doctor examines the child to rule out any contraindications to vaccination. Once your child is eligible for vaccination you will be asked to sign a consent form. The vaccination is carried out by a nurse. After vaccination, observation of the patient is carried out for a minimum of 15 minutes. If there are no worrying symptoms during this time, you can leave the clinic.

6. Where to get the vaccine from?

Compulsory vaccinations are free and available at the POZ. Each year, the Ministry of Health determines which vaccines will be used to implement the vaccination calendar.

If you want to choose a vaccine other than the one chosen by the Ministry you must buy it. You can buy the vaccine at some clinics (you have to ask about it when you register for your appointment) or ask your doctor in advance to write a prescription and buy it at a pharmacy. Vaccines purchased outside the clinic must be stored in your refrigerator on the middle shelf, away from the refrigerator wall, never in the refrigerator door. You need to bring it to the clinic in a heat insulated bag.

7. Which vaccinations are listed as mandatory in Poland?

List of vaccinations from mandatory immunisation programme includes:

- BCG - against tuberculosis
- HepB – Hepatitis B
- RV – Rotavirus
- DTP – Diphtheria, Tetanus, Pertussis
- PCV – Pneumococcal conjugate
- IPV – Inactivated poliovirus
- Hib – Haemophilus influenzae type B
- MMR – Measle, Mumps, Rubella

You can find a vaccination schedule for Poland on this website:

<https://vaccine-schedule.ecdc.europa.eu/Scheduler/ByCountry?SelectedCountryId=166&IncludeChildAgeGroup=true&IncludeChildAgeGroup=false&IncludeAdultAgeGroup=true&IncludeAdultAgeGroup=false>

VII. Organisation of preventive vaccination

1. Where can I get vaccinated?

The vaccination can be carried out at vaccination clinics, most of which are located in POZ.

2. Where can my child be vaccinated?

Vaccination takes place at vaccination centres. During the COVID19 pandemic, a number of population-based vaccination centres were established but the majority of vaccinations, including for children, take place in POZ.

3. What vaccinations should I complete?

Vaccinations are scheduled based on the vaccination calendar, which is updated annually. For children staying in a country for more than 3 months, vaccinations should be completed according to the vaccination calendar of the country concerned. Some vaccinations should be repeated even in adult patients (e.g. against diphtheria, tetanus and whooping cough, against influenza), and their schedule can be agreed with the POZ doctor.

VIII. Night and holiday health care

1. What is night and holiday health care?

When a POZ clinic is closed, the role of POZ is taken over by night and holiday health care clinics (so-called 24-hour care). If you feel unwell and are concerned that your health will deteriorate by the next day but is not life-threatening, go to your nearest night and holiday health care clinic. If you feel unwell but your condition is stable, wait until the next day and see your POZ doctor.

2. What are the hours of night and holiday health care?

From 6pm to 8am and around the clock on public holidays.

3. Where can I find the address of night and holiday health care facilities?

You can find the nearest night and holiday health care clinic on the website of the National Health Fund:

<https://www.nfz.gov.pl/dla-pacjenta/na-ratunek/>.

4. What problems should I bring to the night and holiday healthcare facility?

You can take advantage of night and holiday health care if you are suddenly ill and cannot wait until your POZ clinic is open due to the severity of your symptoms. This does not apply if your life is in danger, in which case you go to SOR.

IX. Casualty department (SOR)

1. What is SOR?

SOR is part of the National Emergency Medical Service. It is designed for patients with life-threatening conditions. In SOR, an initial diagnosis is made and vital functions are stabilised.

2. How long will I wait for help in SOR?

A medical segregation procedure (triage) is carried out on each patient. The time during which aid is granted depends on the category granted:

- red – immediate contact with a doctor
- orange – waiting time for first contact with a doctor up to 10 minutes
- yellow – waiting time for first contact with a doctor up to 60 minutes
- green – waiting time for first contact with a doctor up to 120 minutes
- blue – waiting time for first contact with a doctor up to 240 minutes

In practice, SOR has the highest number of patients in the green and blue groups who should not really be presenting to SOR.

Therefore, waiting times can extend to several hours.

3. What problems can I bring to SOR?

People with a sudden health emergency should come to SOR.

These include injuries (e.g. wounds, joint sprains, contusions suspected to be fractures), burns, frostbite, allergies.

You will also be brought to SOR by the emergency medical service from the scene of an accident or from home if your condition suddenly deteriorates (e.g. persistent shortness of breath, chest pain, loss of consciousness).

4. What problems should not be brought to SOR?

Do not present to SOR for:

- obtaining specialist advice
- writing a prescription
- obtaining a sick note
- changing dressings
- injections
- removing stitches from the wound
- removing plaster dressings
- catheter exchanges
- performance of laboratory tests
- in case of symptoms of infection that are not life-threatening
(do not come in because of a sore throat, runny nose, cough)

5. What documents should I have when coming to SOR?

You need your PESEL number and identity card or passport.

X. Emergency medical service

1. What is an emergency medical service?

The emergency medical service provides assistance in emergencies. For life-threatening emergencies, call the emergency number 999 or 112. The dispatcher will decide on the need for a medical emergency team based on the history taken.

2. When to call an emergency medical service?

You call the emergency medical service if you need help with life-threatening emergencies such as chest pain, paresis, speech impairment, impaired consciousness, traffic accidents.

3. How to call an emergency medical service?

You call an emergency medical service by dialling 112 or 999.

4. Will the emergency medical service take me to a hospital?

The paramedic or doctor will decide whether you should be taken to a hospital. The medical emergency team will decide which SOR you go to – this is usually SOR closest to the scene of the incident, where adequate medical attention can be given.

XI. Outpatient specialist services (AOS)

1. When should I make an appointment for a specialist clinic?

You go to a specialist clinic if a particular health problem cannot be fully diagnosed and treated in the POZ clinic. In most cases you will need a referral from your POZ doctor to make an appointment. In practice, he/she decides at what point you should go to the specialist clinic.

Specialist clinics with a contract with the NFZ do not accept referrals issued by doctors during private consultations.

2. How should I make an appointment for a specialist clinic?

For most outpatient clinics, you will need a referral from your health insurance doctor (a doctor from the POZ or specialist clinic). You receive a 4-digit code, so-called PIN.

It can be printed by your doctor, or you will receive it by SMS or email (if you set this option in IKP).

You can make an appointment by phone or in person. When making an appointment at the clinic, you give this number and your PESEL number.

3. Do I need a referral to every specialist clinic?

No. There are exceptions. You do not need a referral to a psychiatrist, gynaecologist-obstetrician, oncologist, venereologist, dentist.

4. How long is the wait for specialist advice?

The wait for an appointment varies, sometimes many months. If the proposed date is very far away you can ask for a date at another clinic. If you need urgent treatment because of your state of health, the doctor will add this information on the referral (the "pilne" annotation).

You will find information on dates of treatment at specialist clinics, hospitals and surgery on the website:

<https://terminyleczenia.nfz.gov.pl/>

XII. Stay in hospital wards

1. Who can make a referral to a hospital?

You can be referred to the hospital by your health insurance doctor or by a doctor from a private practice.

2. Do I have to agree to the proposed treatment?

You have the right not to consent to hospitalisation. In this case, the doctor will ask you for a written statement of refusal to consent to the proposed treatment.

3. What rights does the patient have?

- The right to information about one's own health. A person of the patient's choice may obtain information about the patient's health. The authorisation may be in writing or oral form.
- The right to refuse treatment
- The right to medical confidentiality
- The right to a supplementary medical opinion
- The right to own medical records. It may also be made available to a statutory representative or a person authorised by the patient.
- The right to be accompanied by a close relative
- The right to pastoral care or the right of direct contact with a priest of the religion professed. You should let the medical staff know that you would like to see a priest of your faith. It

is the task of the medical facility to actually implement this right.

- The right to treatment of pain
- The right to respect for intimacy and personal dignity
- The right to complain

4. Is the hospital obliged to provide an interpreter for a foreign language patient?

The hospital has no legal obligation to provide the patient with an interpreter. Hospitals often create a list of staff members fluent in foreign languages on their own initiative. If you have any problems communicating, please contact the head office or regional branch of NFZ.

XIII. Pregnancy and childbirth

1. Who can carry a pregnancy?

By law, a pregnancy can be carried out by a doctor or midwife. In practice, this is usually a doctor.

2. I am pregnant, when should I see a doctor and do I need a referral?

It is best to see an obstetrician-gynaecologist of your choice by the 10th week of the pregnancy. A visit to the gynaecologist does not require a referral.

3. In what does medical care during pregnancy consist?

The doctor carrying out the pregnancy will order tests according to Polish standards. You can also choose to receive coordinated care for pregnant women (KOC) at a maternity hospital at any stage of your pregnancy. The woman then receives full-profile care during pregnancy, childbirth and for six weeks after the birth of the child. The safety of mum and baby will be supervised by a team consisting of a gynaecologist, midwife and hospital doctors.

4. Am I entitled to an ultrasound scan of my pregnancy?

An ultrasonography is performed at the first visit to confirm the pregnancy. Many doctors perform an ultrasonography at every visit. Around the 12th and 20th weeks of pregnancy, a thorough ultrasonography, known as a genetic ultrasonography, is performed to assess the health of the baby.

5. What are prenatal tests?

Prenatal tests are examinations of the foetus during intrauterine development. The purpose of the tests is to detect congenital diseases or genetic defects. Tests are performed in pregnant women who meet one of the conditions:

- have reached the age of 35
- there was a genetic defect in a previous pregnancy

- the pregnant woman or the father of the child has a known genetic defect
- the doctor has identified a significantly increased risk of having a child affected by a genetic or multifactorial condition
- the result of an ultrasonography or biochemical test performed during pregnancy is abnormal

6. Can I work while the pregnant?

If you feel well you can work during the pregnancy, but you cannot take an overtime. You don't have to work at night (night work is only possible with your permission). If you are worried about your child or your health requires it, you are on sick leave. Any sick leave during pregnancy entitles you to sick pay at 100% of the base.

7. I am pregnant, can my employer dismiss me for this reason?

An employer may not terminate or dissolve an employment contract during pregnancy and maternity leave of an employee.

8. Do pregnant women have special rights?

Yes. Pregnant women are entitled to health care services and pharmaceutical services provided in pharmacies out of turn. "Out of turn" provision in specialist clinics means admission on the day

of registration or at another time, no later than within 7 working days of registration.

Some drugs for pregnant women are free or reimbursed if the doctor mentions this on the prescription (letter B). Free drugs include: insulin, lutein, levothyroxine, metoprolol.

9. Who is a community midwife?

If you are pregnant you should choose a POZ community midwife. You will need to give the details of the POZ midwife and doctor, you choose for your baby at the hospital after the baby is born. The POZ midwife cares for women from 21 weeks of pregnancy and newborns up to 2 calendar months of the child's life. The POZ midwife is required to make 4-6 patronage visits to the child's home. The first visit takes place no later than 48 hours after the mother and child leave the hospital.

The parents should choose a midwife before the baby is born and notify her when the newborn is discharged home.

10. Where in Poland do women give birth?

Women in Poland give birth in hospitals, in maternity wards. There are no birthing rooms in Poland. Home births by choice are rare.

11. What can I do if I have problems with breastfeeding when I return home?

Ask your community midwife for help. There are also lactation clinics at maternity wards, which you can use to help you.

12. What formalities should I do after the birth?

The birth of a child must be reported within 14 days of the birth to the register office (USC). USC will issue the birth certificate, register the child and assign a PESEL number. Once you have received PESEL number, you must register your child for insurance at the workplace of one of the parents or at ZUS. You then need to choose a POZ doctor (paediatrician or family doctor) for your child and arrange a patronage visit up to 4 weeks after the birth of your child (usually 2 weeks after birth).

13. How long does the maternity leave last?

The length of maternity leave depends on the number of children born:

- 20 weeks if one child is born in a single birth,
- 31 weeks if two children are born in a single birth,
- 33 weeks if three children are born in a single birth,

An employee may take up to six weeks of maternity leave before the expected date of childbirth.

If the employee has not taken maternity leave before childbirth, the maternity leave shall start on the day of childbirth.

14. Do I have to take all my maternity leave?

After childbirth, the mother must compulsorily take 14 weeks of leave. She may give up the remaining six weeks of leave and return to work, provided that the remaining maternity leave is

taken by the other carer bringing up the child, who has ceased paid activity in order to provide care.

15. What is the parental leave?

The parental leave serves the purpose of continuing to care for a young child. It is a condition for an employee to take parental leave, irrespective of which parent of the child will be using it, that maternity leave is taken in advance and that the application is submitted to the employer within 21 days of the birth of the child. The parental leave is up to 32 weeks for the birth of one child and up to 34 weeks for the simultaneous birth of more than one child. The parental leave may be taken:

- entirely by one of the employees-parents,
- by both employees-parents at the same time, but then the periods of leave taken by the employee-mother and the employee-father are added together and may not exceed 32 weeks for one child and 34 weeks for more than one child born at the same time
- alternating between the two parents, who will share the 32 weeks.

16. Will my pay change during my maternity and parental leave?

You are entitled to be paid 80% of your salary for the whole year or 100% for 20 weeks, and 60% on your parental leave.

17. What is the childcare leave?

The childcare leave is granted for the personal care of a child to an employee who has been employed for at least 6 months. Previous periods of employment are also included in this period of employment. The childcare leave can be taken by both mother and father if both are employees.

This leave lasts for a maximum of 36 months. It can be divided into up to 5 parts. You can use it until the end of the calendar year in which your child turns 6. In the case of the childcare leave to care for a disabled child, you can take it until the child turns 18.

XIV. Dental care

1. Do I need a referral to go to the dentist?

You do not need a referral to visit a dentist (dental practitioner).

2. Why do Poles choose private dental care?

Poles are opting for private dental care due to the significantly reduced reimbursable benefits. White fillings are only reimbursed in teeth 1 to 3, in the case of adults these are chemically-cured fillings, in the case of children they are light-cured. Reimbursed root canal treatment is only available for teeth 1 to 3. Orthodontic treatment with removable braces only applies until the age of 12. Significant limitations also apply to prosthetics. You can find the list of reimbursed services at:

<http://www.nfz-warszawa.pl/dla-pacjenta/co-kazdy-pacjent-wiedziec-powinien/leczenie-stomatologiczne/>

3. What to do in case of a sudden toothache?

In the case of a sudden toothache, you should be admitted to a clinic with a NFZ contract on the same day. If you have a toothache when clinics are closed (at night or on a public holiday) go to an emergency dental service.

You can find a list of facilities at:

<https://pacjent.gov.pl/dorazna-pomoc-dentystyczna>

XV. Medical rehabilitation

1. With which medical conditions can I apply for rehabilitation?

Medical rehabilitation is a method of treatment for many diseases of the musculoskeletal system, the nervous system, the circulatory system and the respiratory system. It is used after surgical operations, including oncological ones.

2. What does rehabilitation treatment look like?

A referral from your POZ doctor is required for rehabilitation.

Rehabilitation can take place:

- ambulatory – for patients requiring rehabilitation or physiotherapy who move independently
- at home – for patients requiring rehabilitation or physiotherapy who not move independently

- in a centre or day unit – for patients whose health condition does not allow rehabilitation in an ambulatory setting, but who do not require round-the-clock medical supervision
- stationary – for patients who require complex rehabilitation services and round-the-clock medical and nursing supervision due to continuing treatment.

For more information, visit:

<https://www.gov.pl/web/zdrowie/rehabilitacja-lecznicza>

3. What is rehabilitation benefit?

The rehabilitation benefit is a kind of cash benefit for persons who have used up all their sick leave due to illness and are still unable to work, and further medical treatment or rehabilitation is likely to restore their ability to work within a period of up to 12 months. It is paid by ZUS or by the employer.

XVI. Spa treatment

1. What is spa treatment?

Spa treatment is a continuation of inpatient or outpatient treatment. If you require certain treatments at spas in order to recover from an illness or accident, or because of an occupational disease, you may benefit from this type of treatment. The spa uses mineral waters, mud, climatotherapeutic stimuli, hydrotherapy, physical therapy, kinesitherapy.

Only patients who are fit enough to travel to the spa and who are independent may be referred for spa treatment.

2. Is a referral required for spa treatment?

A referral for spa treatment is issued by your health insurance doctor, taking into account your current state of health.

3. I have received a referral for spa treatment and what next?

The referral is sent by the doctor or by you to the regional branch of NFZ. It should be sent within 30 days of the date of issue, otherwise it is no longer valid. It is then checked by a doctor employed by the Fund, who assesses its advisability and indicate the type and location of spa treatment.

If the specialist determines that there are contraindications or no indications for treatment, he/she does not confirm the referral. It will be returned to the doctor who issued it and you will receive written information. The decision is final and cannot be appealed. If a date of treatment is agreed, the regional branch of NFZ will provide you with a confirmed referral up to 14 days before the date of treatment.

4. How long does spa treatment last?

For an adult, a stay in a spa hospital lasts 21 days. The child's stay lasts 27 days. Such a stay is free of charge.

The stay in the spa sanatorium lasts 21 days. The stay of a child is free of charge, while the stay of an adult is partially charged.

Stay in a spa hospital for spa rehabilitation lasts 28 days and is free of charge.

Stay in a spa sanatorium for spa rehabilitation lasts 28 days and is partially charged.

Ambulatory spa treatment of adults and children lasts from 6 to 18 days.

For more information, visit: <https://www.nfz.gov.pl/dla-pacjenta/leczenie-uzdrowiskowe1/>

XVII. Addiction treatment

1. What should I do if I suspect I have an addiction?

It is best to visit your POZ doctor who will refer you to the appropriate clinic or hospital. You can also see a psychiatrist straight away, to whom you do not need a referral. If you are addicted to alcohol or psychotropic substances you do not need a referral to drug treatment facilities.

2. What does drug treatment look like?

Drug treatment for addiction is voluntary, except where there is a court obligation to treat. Treatment can be inpatient in a hospital or ambulatory.

You can find a list of addiction treatment facilities at <https://swiatprzychodni.pl/swiadczenia/leczenie-uzaleznien/>

3. Where can I find information on treatment for alcohol addiction?

You can find a list of facilities and other important information on the website of the State Agency for the Solution of Alcohol Problems: <https://www.parpa.pl/>

4. What is a klub AA?

These are the Alcoholics Anonymous clubs, self-help centres set up by sober people. They live on their own contributions. They are located in many towns, including smaller ones.

You can find a list of AA meetings on the website: <https://spis.aa.org.pl/>

XVIII. Oncology

1. What should I do if I suspect I have cancer?

The best thing to do is to see your POZ doctor, who will carry out appropriate tests if necessary, or refer you to an oncologist. You can also go directly to an oncology clinic, to which you do not need a referral.

2. What is a karta DiLO?

With a karta DiLO card, a patient with a suspected cancer enters the path of rapid cancer diagnosis and treatment. The karta DiLO card can be issued by a GP, a doctor at a specialist clinic or in hospital.

3. Does the insurance provide palliative and hospice care?

If you have terminal cancer and have been referred by a doctor, you are entitled to receive care in an inpatient hospice or palliative care unit, a home hospice or a palliative care clinic. Hospices aim to improve the quality of life of patients in terminal illness and are also located in smaller towns.

A list of facilities can be found on the regional NFZ website under "opieka paliatywna".

XIX. Medical products

1. What are medical products?

Medical products include prostheses, orthopaedic appliances, wheelchairs, canes, crutches, walkers, orthopaedic footwear, infusion sets for personal insulin pumps, nappies, catheters, ostomy equipment, hearing aids, spectacle lenses, anti-decubitus mattresses and prosthetic limbs.

2. Are medical products reimbursed?

Patients are entitled to reimbursement for medical products. Reimbursement means that NFZ can partially or fully cover the cost of a medical product.

3. How do I get an order for medical products?

The order for medical products is usually issued by a doctor, but also by a nurse, midwife, physiotherapist or other authorised persons.

4. Where can I buy medical products?

You can buy medical products at points which have an agreement with NFZ for the provision of medical products. These are usually medical shops or pharmacies.

XX. Trusted profile

1. What is a trusted profile?

The trusted profile is a confirmed set of data which uniquely identifies a person in electronic services provided by offices. These data are name, surname, date of birth and PESEL number. The trusted profile is secured in such a way that no one but its owner can use it.

2. How do I set up my trusted profile?

You can set up the trusted profile on the government website

<https://www.gov.pl/web/gov/zaloz-profil-zaufany>.

Instructions on how to set up the trusted profile can be found on the government website, unfortunately it is only available in Polish

<https://psz.praca.gov.pl/documents/10240/11841826/Instrukcja%20jak%20za%C5%82o%C5%BCy%C4%87%20Profil%20Zaufany.pdf/8d8c2a5d-a62a-4428-9d64-61c3e9917934/>

XXI. Glossary

Prenatal tests – tests performed to detect congenital diseases or genetic defects in the foetus.

Well-child care – preventive check-ups for children at 2, 4, 6, 9, 16 and 18 years of age.

Electronic Platform of Public Administration Services

(ePUAP) – a platform that makes it possible to deal with many official matters from home. Documents submitted electronically can be signed using a trusted profile.

Online patient account (IKP) – an online platform that brings together patient information in one place and allows a range of issues to be dealt with without the need for a visit to a clinic.

Card for Cancer Diagnosis and Treatment (DiLO) – a document issued by a POZ doctor or specialist enabling patients with suspected cancer to be diagnosed and treated quickly.

Drug treatment – a form of treatment focusing on the fight against addiction. It takes place in the form of individual and group therapies or in specially adapted inpatient centres.

Rehabilitation treatment - procedures aimed at improving the patient's physical and mental functioning. A method of treatment for many diseases of the musculoskeletal system, the nervous system, the circulatory system and the respiratory system.

Spa treatment – a form of treatment in spa centres that includes treatments to improve health, comprehensive specialist care and health education.
Primary Health Care (POZ) doctor – a

general practitioner. A family medicine doctor, internist or paediatrician, who practices in a POZ clinic.

National Health Fund (NFZ) – a state organisational unit which, in the system of universal health care, deals with the financing of health services.

Night and holiday health care – takes over the role of POZ facilities after their closure. It is open from 6pm to 8am and around the clock on public holidays. The term 24-hour care is often used interchangeably.

Primary Health Care (POZ) nurse – a community nurse caring for patients in homes and clinics.

Emergency medical service – an organisational unit within the health care system providing emergency care. For life-threatening emergencies, call the emergency number 999 or 112.

Primary Health Care (POZ) midwife - cares for women from 21 weeks of pregnancy and newborns up to 2 calendar months of the child's life.

Specialist clinic – the place where services are provided by specialist doctors. To go to most specialist clinics, you need a referral from your POZ doctor.

Primary Health Care (POZ) - a part of the health care system providing all persons entitled to benefits residing/staying in the territory of Poland with health care services close to their place of residence.

Trusted profile - is a confirmed set of data which uniquely identifies a person in electronic services provided by offices.

Annual prescription – a prescription valid for 365 days, on which the doctor writes more packs of medicines, which the patient can gradually pick up at the pharmacy.

Ombudsman for Patients' Rights – a body established to ensure that the rights of patients are respected by clinics, dispensaries, hospitals and doctors and to deal with their requests and complaints.

Casualty department (SOR) – a part of the National Emergency Medical Service for patients with a life-threatening condition or who have suffered an emergency.

Rehabilitation benefit – a cash benefit paid by ZUS or the employer to people who have used up all their sick leave and are still unable to work.

Health services - activities to preserve, save, restore and improve health, e.g. visiting a doctor, lab tests, surgery

Beneficiary - a person entitled to health services financed from public funds.

Service provider – an entity providing healthcare services that has a contract with NFZ, e.g. a primary health care clinic.

Online doctor consultation – a telephone consultation with a doctor.

Health insurance – entitles the insured person to health services financed from public funds.

Maternity leave – compulsory leave for every woman with an employment contract after the birth of her child.

Parental leave - serves the purpose of continuing to care for a young child. It applies to both men and women. It is conditional on taking the maternity leave.

Childcare leave - is granted for the personal care of a child to an employee who has been employed for at least 6 months.

Medical products - a group of products for medical purposes with strictly defined rules for design, sale and use.

Social Insurance Institution (ZUS) – a state organisational unit carrying out tasks related to social insurance in Poland.

XXII. Forms of declaration of choice of doctor, nurse and midwife of the POZ

DECLARATION OF CHOICE OF PRIMARY HEALTH CARE PROVIDER AND PRIMARY HEALTH CARE DOCTOR

1. DATA RELATING TO THE BENEFICIARY												
1. Forename						2. Surname						
3. Date of birth				4. PESEL number or, failing that, series and number of the identity document								
day - month - year												
5. Address of the place of residence												
5A. Street						5B. House / flat number			5C. Postcode and town			
6. Phone number (optional field)												
7. E-mail address (optional field)												
8. Data concerning the statutory representative (if the beneficiary is a minor or totally incapacitated person ¹⁾)												
8A. Forename		8B. Surname				8C. Phone number (optional field)						
Address of the place of residence												
8D. Street				8E. House / flat number			8F. Postcode and town					
8G. Forename		8H. Surname				8I. Phone number (optional field)						
Address of the place of residence												
8J. Street				8K. House / flat number			8L. Postcode and town					

II. DATA RELATING TO THE PROVIDER	
9. Pursuant to Article 9(1) of the Act of 27 October 2017 on Primary Health Care (Journal of Laws of 2020, item 172), I declare my choice:	
9A. Name (company) of the service provider	
9B. Registered address of the service provider	
10. In the current calendar year I am choosing: ²⁾	
☐ for the first or second time	☐ for the third and subsequent time ³⁾
11. If the selection is made for the third or subsequent time in the current calendar year, indicate whether the reason for the selection is:	
☐ the circumstance set out in Article 9(5) of the Act of 27 October 2017 on primary health care: ☐ a change of place of residence - o cessation of health care services by the selected provider, the primary health care doctor at the selected provider - o attaining the age of 18 by the beneficiary, if the primary health care doctor is a doctor with first or second degree specialization or a specialist title in the field of pediatrics - o for other reasons attributable to the provider (please specify) ☐ other circumstance	
III. DATA RELATING TO THE PRIMARY HEALTH CARE DOCTOR	
12. Pursuant to Article 9(2) of the Act of 27 October 2017 on primary health care, I declare my choice of: ⁴⁾	
Name of the primary health care doctor	
13. In the current calendar year I am choosing: ²⁾	
☐ for the first or second time	☐ for the third and subsequent time ³⁾
14. If the selection is made for the third or subsequent time in the current calendar year, indicate whether the reason for the selection is:	
☐ the circumstance set out in Article 9(5) of the Act of 27 October 2017 on primary health care: ☐ a change of place of residence - o cessation of health care services by the selected provider, the primary health care doctor at the selected provider - o attaining the age of 18 by the beneficiary, if the primary health care doctor is a doctor with first or second degree specialization or a specialist title in the field of pediatrics - o for other reasons attributable to the provider (please specify) ☐ other circumstance	
..... (date)	 (signature of the beneficiary or legal representative) ⁵⁾

EXPLANATIONS:

- 1)
Declarations made by statutory representatives on behalf of minors after they reach the age of majority remain valid until another primary care doctor is selected, and in a situation where the primary care doctor is a doctor with a first or second degree specialisation or a specialist title in paediatrics, the declarations remain valid with regard to the selection of the provider until another primary health care doctor is selected or a new provider is selected (Article 6(3) and Article 10(7) of the Act of 27 October 2017 on Primary Health Care).
- 2)
The beneficiary has the right to choose a provider or a primary health care doctor free of charge not more than twice in a calendar year, and in case of every subsequent change he/she pays a fee of 80 PLN. The beneficiary does not pay the fee in the case of a change of his/her place of residence or in the case of cessation of the provision of health care services by the selected provider, a primary health care doctor at the selected provider, or in a situation where the beneficiary, whose primary health care doctor is a doctor with a first or second degree specialisation or a specialist title in paediatrics, has reached the age of 18, or for other reasons arising on the side of the provider (Article 9(4) and (5) of the Act of 27 October 2017 on Primary Health Care).
- 3)
In the case of the third and subsequent choice, information should be provided on whether the reason for the choice is a circumstance specified in Article 9(5) of the Act of 27 October 2017 on Primary Health Care, i.e. a change of the beneficiary's place of residence or cessation of the provision of health care services by the selected provider, a primary health care doctor at the selected provider or a change of the primary health care doctor, in a situation where the beneficiary, whose primary health care doctor is a doctor with a first or second degree specialisation or a specialist title in paediatrics, has reached 18 years of age, or for other reasons on the side of the provider.
- 4)
A beneficiary can choose a primary health care doctor, a primary health care nurse or a primary health care midwife from the same provider, from different providers or from being providers (Article 9(3) of the Act of 27 October 2017 on Primary Health Care).
- 5)
To be completed in the case of a declaration submitted in paper form or electronic form, referred to in Article 10(1)(1) or (2) of the Act of 27 October 2017 on Primary Health Care.

DECLARATION OF CHOICE OF PRIMARY HEALTH CARE PROVIDER AND PRIMARY HEALTH CARE NURSE

1. DATA RELATING TO THE BENEFICIARY												
1. Forename						2. Surname						
3. Date of birth				4. PESEL number or, failing that, series and number of the identity document								
day - month - year												
5. Address of the place of residence												
5A. Street						5B. House / flat number			5C. Postcode and town			
6. Phone number (optional field)												
7. E-mail address (optional field)												
8. Data concerning the statutory representative (if the beneficiary is a minor or totally incapacitated person ¹⁾)												
8A. Forename			8B. Surname			8C. Phone number (optional field)						
Address of the place of residence												
8D. Street				8E. House / flat number			8F. Postcode and town					
8G. Forename			8H. Surname			8I. Phone number (optional field)						
Address of the place of residence												
8J. Street				8K. House / flat number			8L. Postcode and town					

II. DATA RELATING TO THE PROVIDER

9. Pursuant to Article 9(1) of the Act of 27 October 2017 on Primary Health Care (Journal of Laws of 2020, item 172), I declare my choice:

9A. Name (company) of the service provider

9B. Registered address of the service provider

10. In the current calendar year I am choosing:²⁾

☐ for the first or second time

☐ for the third and subsequent time³⁾

11. If the selection is made for the third or subsequent time in the current calendar year, indicate whether the reason for the selection is:

- ☐ the circumstance set out in Article 9(5) of the Act of 27 October 2017 on primary health care: ☐ a change of place of residence
- ☐ cessation of health care services by the selected provider, the primary health care nurse at the selected provider
 - ☐ for other reasons attributable to the provider (please specify)
 -
 -
- ☐ other circumstance

III. DATA RELATING TO THE PRIMARY HEALTH CARE NURSE

12. Pursuant to Article 9(2) of the Act of 27 October 2017 on primary health care, I declare my choice of:⁴⁾

Name of the primary health care nurse

13. In the current calendar year I am choosing:²⁾

☐ for the first or second time

☐ for the third and subsequent time³⁾

14. If the selection is made for the third or subsequent time in the current calendar year, indicate whether the reason for the selection is:

- ☐ the circumstance set out in Article 9(5) of the Act of 27 October 2017 on primary health care: ☐ a change of place of residence
- ☐ cessation of health care services by the selected provider, the primary health care nurse at the selected provider
 - ☐ for other reasons attributable to the provider (please specify)
 -
 -
- ☐ other circumstance

.....
(date)

.....
(signature of the beneficiary or legal representative)⁵⁾

EXPLANATIONS:

- 1)
Declarations made by legal representatives on behalf of minors after they have reached the age of majority shall remain valid until a new primary health care nurse is chosen.
- 2)
The beneficiary has the right to choose a provider or a primary health care nurse free of charge not more than twice in a calendar year, and in case of every subsequent change he/she pays a fee of 80 PLN. The beneficiary does not pay the fee in the case of a change of his/her place of residence or in the case of cessation of the provision of health care services by the selected provider, the primary health care nurse at the selected provider, or for other reasons arising from the provider's side (Article 9(4) and (5) of the Act of 27 October 2017 on Primary Health Care).
- 3)
In the case of the third and subsequent choice, information must be provided as to whether the reason for the choice is a circumstance specified in Article 9(5) of the Act of 27 October 2017 on Primary Health Care, i.e. a change in the beneficiary's place of residence or cessation of the provision of health care services by the selected provider, a primary health care nurse at the selected provider, or for other reasons on the part of the provider.
- 4)
A beneficiary can choose a primary health care doctor, a primary health care nurse or a primary health care midwife from the same provider, from different providers or from being providers (Article 9(3) of the Act of 27 October 2017 on Primary Health Care).
- 5)
To be completed in the case of a declaration submitted in paper form or electronic form, referred to in Article 10(1)(1) or (2) of the Act of 27 October 2017 on Primary Health Care.

DECLARATION OF CHOICE OF PRIMARY HEALTH CARE PROVIDER AND PRIMARY HEALTH CARE MIDWIFE

1. DATA RELATING TO THE BENEFICIARY												
1. Forename						2. Surname						
3. Date of birth				4. PESEL number or, failing that, series and number of the identity document								
day - month - year												
5. Address of the place of residence												
5A. Street						5B. House / flat number			5C. Postcode and town			
6. Phone number (optional field)												
7. E-mail address (optional field)												
8. Data concerning the statutory representative (if the beneficiary is a minor or totally incapacitated person ¹⁾)												
8A. Forename			8B. Surname			8C. Phone number (optional field)						
Address of the place of residence												
8D. Street				8E. House / flat number			8F. Postcode and town					
8G. Forename												
8H. Surname			8I. Phone number (optional field)									
Address of the place of residence												
8J. Street				8K. House / flat number			8L. Postcode and town					

II. DATA RELATING TO THE PROVIDER			
9. Pursuant to Article 9(1) of the Act of 27 October 2017 on Primary Health Care (Journal of Laws of 2020, item 172), I declare my choice:			
	9A. Name (company) of the service provider		
	9B. Registered address of the service provider		
10. In the current calendar year I am choosing: ²⁾			
	☐ for the first or second time	☐ for the third and subsequent time ³⁾	
11. If the selection is made for the third or subsequent time in the current calendar year, indicate whether the reason for the selection is:			
☐ the circumstance set out in Article 9(5) of the Act of 27 October 2017 on primary health care: ☐ a change of place of residence ☐ cessation of health care services by the selected provider, the primary health care midwife at the selected provider ☐ for other reasons attributable to the provider (please specify) ☐ other circumstance			
III. DATA RELATING TO THE PRIMARY HEALTH CARE MIDWIFE			
12. Pursuant to Article 9(2) of the Act of 27 October 2017 on primary health care, I declare my choice of: ⁴⁾			
	Name of the primary health care midwife		
13. In the current calendar year I am choosing: ²⁾			
	☐ for the first or second time	☐ for the third and subsequent time ³⁾	
14. If the selection is made for the third or subsequent time in the current calendar year, indicate whether the reason for the selection is:			
☐ the circumstance set out in Article 9(5) of the Act of 27 October 2017 on primary health care: ☐ a change of place of residence ☐ cessation of health care services by the selected provider, the primary health care midwife at the selected provider ☐ for other reasons attributable to the provider (please specify) ☐ other circumstance			
..... (date)		 (signature of the beneficiary or legal representative) ⁵⁾	

EXPLANATIONS:

- 1)
Declarations made by legal representatives on behalf of minors after they have reached the age of majority shall remain valid until a new primary health care midwife is chosen.
- 2)
The beneficiary has the right to choose a provider or a primary health care midwife free of charge not more than twice in a calendar year, and in case of every subsequent change he/she pays a fee of 80 PLN. The beneficiary does not pay the fee in the case of a change of his/her place of residence or in the case of cessation of the provision of health care services by the selected provider, the primary health care midwife at the selected provider, or for other reasons arising from the provider's side (Article 9(4) and (5) of the Act of 27 October 2017 on Primary Health Care).
- 3)
In the case of the third and subsequent choice, information must be provided as to whether the reason for the choice is a circumstance specified in Article 9(5) of the Act of 27 October 2017 on Primary Health Care, i.e. a change in the beneficiary's place of residence or cessation of the provision of health care services by the selected provider, a primary health care midwife at the selected provider, or for other reasons on the part of the provider.
- 4)
A beneficiary can choose a primary health care doctor, a primary health care nurse or a primary health care midwife from the same provider, from different providers or from being providers (Article 9(3) of the Act of 27 October 2017 on Primary Health Care).
- 5)
To be completed in the case of a declaration submitted in paper form or electronic form, referred to in Article 10(1)(1) or (2) of the Act of 27 October 2017 on Primary Health Care.

XXIII. Patronage consultations and balance tests, including screening tests

No	AGE OR STAGE OF EDUCATION	PATRONAGE CONSULTATION AND BALANCE TESTS, INCLUDING SCREENING TESTS*
1	1-4 weeks of age	Patronage consultation from a primary care physician, including taking of medical history and physical examination, taking into account a physical development, measurement and monitoring of head circumference, evaluation of jaundice, basic neurological assessment, and physical examination for the detection of congenital hip dysplasia
2	2-6 months of age (on dates corresponding to preventive vaccinations)	• Taking medical history and physical examination, taking into account physical development (measurements: body weight and length, head and chest circumference), past diseases and prevention of rickets. • Assessment of the size of the anterior fontanel. • Assessment of the neurological state. • Determination of the dental age. • In boys, examination of the presence of testicles in the scrotum. • Conducting a test of the development of auditory reactions - in the case of an incorrect test result or a diagnosis of belonging to the risk group of hearing damage, referral to a specialist

		audiological or phoniatric diagnostics. • Assessment of the presence of risk factors for hearing damage. • Physical examination to detect congenital hip dysplasia.
3	9 months of age	• Taking medical history and physical examination, taking into account the pace of physical development (measurements: body weight and length, head and chest circumference) and psychomotor development. • Assessment of the implementation of preventive vaccinations in accordance with the vaccination schedule. • Conducting a test of the development of auditory reactions.
4	12 months of age	• Taking medical history and physical examination, taking into account the pace of physical development (measurements: body weight and length, head and chest circumference) and psychomotor development. • Assessment of the implementation of preventive vaccinations in accordance with the vaccination schedule. • Conducting a test of the development of auditory reactions. • Assessment of the presence of new risk factors for hearing damage. • Measurement of blood pressure.
5	2 years	Preventive medical examination (health

		balance) includes: • taking medical history and physical examination, with particular emphasis on the pace of physical development (measurements: body weight and length), including the determination of the Body Mass Index (BMI), and psychomotor development as well as past diseases; • evaluation of the implementation of preventive vaccinations in accordance with the vaccination schedule; • assessment of the presence of risk factors for hearing damage; • conducting a test of the development of auditory reactions and assessment of speech development; • Hirschberg test for strabismus detection; • blood pressure measurement.
6	4 years	Preventive medical examination (health balance) includes: • assessment of the presence of risk factors for hearing damage; • taking medical history and physical examination to assess physical development (measurements: body height and weight), including the determination of the Body Mass Index (BMI), psychomotor and social development;

		- conducting a test of the development of auditory reactions and assessment of speech development; - oral hygiene assessment.
7	5 years	Preventive medical examination (health balance) includes: - taking medical history and physical examination to assess physical development (measurements: body height and weight), including the determination of the Body Mass Index (BMI), psychomotor and social development; - blood pressure measurement.
8	One-year compulsory pre-school preparation*** (****)	Preventive medical examination (health balance) includes: - taking of medical history: - an interview with statutory representatives or legal or actual guardians and the child, taking into account health risk factors and health behaviors, - analysis of the information contained in the preventive examination card, - analysis of other available individual medical records; - physical examination with particular emphasis on the assessment of: a) physical development (measurements: body height and weight), including the determination of the Body Mass Index

		(BMI), • psychomotor development, • speech, • laterality, • the locomotor system, • the oral cavity, • in boys - the presence of testicles in the scrotum and defects of the genitourinary system; • detection of locomotor system disorders, including scoliosis, static deformations of the lower limbs; • trabismus detection (Cover test, Hirschberg test); • detection of visual acuity disorders; • evaluation of the implementation of preventive vaccinations in accordance with the vaccination schedule; • conducting a test of the development of auditory reactions and assessment of speech development; • assessment of the presence of risk factors for hearing damage; • blood pressure measurement; • specialist medical and diagnostic examination, if necessary; • summary of the examination, specifying health school maturity and qualification for a group in physical education (including swimming lessons) and a
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		possible health problem; counseling in the field of a healthy lifestyle.
9	3rd grade of primary school****	Preventive medical examination (health balance) includes: - taking of medical history: - an interview with statutory representatives or legal or actual guardians and the child, taking into account health risk factors and health behaviors, - analysis of the information contained in the preventive examination card from the school nurse or hygienist, with particular emphasis on the results of screening tests, and from the class teacher, - analysis of other available individual medical records; - physical examination with particular emphasis on the assessment of: - physical development (measurements: body height and weight), including the determination of the Body Mass Index (BMI), - psychosocial development, - the locomotor system, - genitourinary system, - sexual maturation according to the Tanner scale,

		• thyroid gland, • the oral cavity; • detection of locomotor system disorders, including scoliosis; • detection of visual acuity and color vision disorders; • blood pressure measurement; • specialist medical and diagnostic tests, if necessary; • summary of the examination, specifying the level and pace of physical development, psychosocial development (indicative assessment), school adaptation, qualification for a group in physical education and school sports, and a possible health problem; • counseling in the field of a healthy lifestyle.
10	1st grade of lower secondary school****	Preventive medical examination (health balance) includes: • taking of medical history: • an interview with statutory representatives or legal or actual guardians and the child, taking into account health risk factors and health behaviors, • analysis of the information contained in the preventive examination card from the school nurse or hygienist, with particular emphasis on the

		results of screening tests, and from the class teacher, • analysis of other available individual medical records; • physical examination with particular emphasis on the assessment of: • physical development (measurements: body height and weight), including the determination of the Body Mass Index (BMI), • psychosocial development, • the locomotor system, • thyroid gland, • sexual maturation, • genitourinary system, • the oral cavity, • skin; • detection of locomotor system disorders, including scoliosis and excessive thoracic kyphosis; • detection of visual acuity disorders; • detection of hearing disorders (orientation test in a whisper); • blood pressure measurement; • specialist medical and diagnostic tests, if necessary; • summary of the examination, specifying the level and rate of growth and sexual maturation, psychosocial development (indicative assessment), school
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		adaptation, qualifications to the group in physical education and school sports, and a possible health problem; counseling in the field of a healthy lifestyle.
11	1st grade of upper secondary school****	Preventive medical examination (health balance) includes: - taking medical history: - an interview from the student and statutory representatives or legal or actual guardians, taking into account health risk factors and health behaviors, - analysis of the information contained in the preventive examination card from the school nurse or hygienist, with particular emphasis on the results of screening tests, and from the class teacher, - analysis of other available individual medical records; - physical examination with particular emphasis on the assessment of: - physical development (measurements: body height and weight), including the determination of the Body Mass
12	The last year of upper secondary school until the age of 19	Preventive medical examination (health balance) includes: - taking medical history: - interview from the student, taking

		into account health risk factors and health behaviors, • analysis of the information contained in the preventive examination card from the school nurse or hygienist, with particular emphasis on the results of screening tests, and from the class teacher, • analysis of other available individual medical records; • physical examination with particular emphasis on the assessment of: • physical development (measurements: body height and weight), including the determination of the Body Mass Index (BMI), • psychosocial development, • genitourinary system and sexual maturity, • the locomotor system, • thyroid gland, • the oral cavity, • skin; • detection of visual acuity disorders; • blood pressure measurement; • specialist medical and diagnostic tests, if necessary; • summary of the results of the examination, specifying a possible health problem as well as suggestions and
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		advice regarding further education and choice of profession, future parenthood, healthy lifestyle, including physical activity.
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* Examinations in children up to 16 years of age are performed in the presence of legal or actual guardians.

** For a 5-year-old child who is not completing the compulsory one-year pre-school preparation.

*** If the examination is not performed on time, screening tests are performed in the 1st grade of primary school.

**** In the absence of promotion to the next grade, screening tests are not performed again.